A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

01 May 2026

Dr Amanda Richards
Pinnacle Surgery

Dear Dr Richards,

Re: Mr Colin Scheggia
DOB: 07/02/1976 Phone: 0407 483 980
Address: 207 Horward Street, SOLDIERS HILL, VIC, 3350
Email: colin.scheggia@hotmail.com
Medicare: 3422023406 / 1

Summary

- **Significant improvement in gastrointestinal symptoms and chronic joint pain on low-dose amitriptyline.**
- **Oesophageal manometry excluding a primary motility disorder.**
- **Escalation of amitriptyline dose for symptom optimisation.**
- **Commencement of pantoprazole for visceral hypersensitivity and reflux.**
- **No action required by you.**

I had the pleasure of conducting a review with Colin Scheggia to discuss his response to treatment and recent investigation results. Colin has commenced amitriptyline and has successfully titrated his dose to 20 mg daily. He reports a significant clinical response, estimating a 30% to 60% improvement in his overall pain levels.

His bowel habit has regularised significantly; whereas he previously reported seven to eight movements per day, he is now opening his bowels zero to two times daily with improved consistency. There has been a concomitant reduction in urgency and anal leakage. Interestingly, he also noted the resolution of his bruxism and a marked improvement in his chronic joint and nerve pain. While he experienced some initial morning lethargy, he has successfully mitigated this by taking his dose earlier in the evening.

Regarding his upper gastrointestinal symptoms, he reports that his dysphagia to solids, specifically rice and sushi, has lessened in severity, and he has not experienced any further vomiting. We reviewed his oesophageal manometry, which was unremarkable and excluded any primary motility disorders.

Impression and Plan

The current strategy is to optimise the neuromodulatory benefits of amitriptyline. I have encouraged Colin to slowly escalate his dose as tolerated, with a target of 50 mg daily. I have provided a prescription for 25 mg tablets to assist with this titration.

Furthermore, I have commenced him on pantoprazole 40 mg daily, to be taken before breakfast. My conceptualisation of his condition involves significant visceral hypersensitivity; while antacids may have been ineffective in the past, I believe the amitriptyline may now allow the pantoprazole to provide better

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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symptomatic relief by reducing the sensory triggers in the oesophageal mucosa.

We discussed the potential for constipation as the amitriptyline dose increases, and I have advised the use of Osmolax or kiwi fruit as required. I have opted to defer dietary reintroduction and pelvic floor physiotherapy for the time being, as we first want to establish his optimal dose of amitriptyline without introducing confounding variables. I will review Colin via telehealth in three to four months.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Adam Girardin, Dr Emily Martin

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